

DEPARTMENT OF PATHOLOGY

ICD-0-3
papillary,
CQCF: Carcinoma, follicular variant 8340/3
Path: Carcinoma, papillary thyroid 8260/3
Site: thyroid, NOS C73.9
3/6/11 *lw*

Surgical Pathology Consultation Report

Patient

Name:

MRN:

DOB:

Service:

Visit #:

Accession #:

Collected:

Received:

Gender: F

Location:

Reported:

HCN:

Facility:

Ordering

MD:

Copy To:

Specimen(s) Received

1. Thyroid: left hemi stitch marks upper pole-for tumor banking

Diagnosis

Papillary carcinoma, 2.7 cm, oncocytic variant, with follicular nodular disease and follicular adenoma, 2.5cm: Thyroid

Double adenoma, one chief cell type and one clear cell variant: Parathyroid

- Left hemithyroidectomy specimen

Synoptic Data

Procedure: Left hemithyroidectomy
Received: Fresh
Specimen Integrity: Intact
Specimen Size: Left lobe:
6.1 cm
2.7 cm
2.0 cm
Isthmus +/- pyramidal lobe:
1.6 cm
0.7 cm
0.5 cm

Specimen Weight: 20.6 g
Tumor Focality: Unifocal

----- DOMINANT TUMOR -----

Tumor Laterality: Left lobe
Tumor Size: Greatest dimension: 2.7cm
Additional dimension: 2.6 cm
Additional dimension: 2 cm

Histologic Type: Papillary carcinoma, oncocytic or oxyphilic variant
Follicular architecture
Oncocytic or oxyphilic cytomorphology

Histologic Grade: Not applicable

Margins: Margins uninvolved by carcinoma
Distance of invasive carcinoma to closest margin: 0.1mm

Tumor Capsule: Partially encapsulated

Tumor Capsular Invasion: Present, extent minimal

UUID:807E51BC-B961-4B1C-81A1-352CED88468A
TCGA-EM-A1YB-01A-PR

Redacted



PW JSS 2/2013, TCGA tumor is 100% follicular variant. lw

Criteria	Yes	No
Diagnosis Discrepancy	☒	☐
Primary Tumor Site Discrepancy	☒	☐
HIPAA Discrepancy	☒	☐
Prior Malignancy History	☒	☐
Dual/Synchronous Primary Noted	☒	☐
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	<i>KML</i>	
Date Reviewed	<i>2/6/11</i>	

Lymph-Vascular Invasion:	Not identified
Perineural Invasion:	Not identified
Extrathyroidal Extension:	Not identified
TNM Descriptors:	Not applicable
Primary Tumor (pT):	pT2: Tumor more than 2 cm, but not more than 4 cm, limited to thyroid
Regional Lymph Nodes (pN):	pNX: Regional lymph nodes cannot be assigned Number of regional lymph nodes examined: 0 Number of regional lymph nodes involved: 0
Distant Metastasis (pM):	Not applicable
Additional Pathologic Findings:	Adenoma Adenomatoid nodule(s) or Nodular follicular disease Thyroiditis, focal (nonspecific) Parathyroid gland(s), other: double adenomas

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Comment

The parathyroid contains two prominent nodules, one composed of chief cells and the other composed of clear cells. These nodules each have their own capsule and there is an adjacent rim of compressed atrophic parathyroid, consistent with these nodules being adenomas.

benign

Electronically verified

by:

MD

Gross Description

The specimen labeled with the patient's name and as "left hemi stitch marks upper pole" consists of a left hemithyroidectomy specimen that is oriented with a suture and weighs 20.6 g. The lobe measures 6.1 cm SI x 2.7 cm ML x 2 cm AP and the isthmus measures 1.6 SI x 0.7 cm ML x 0.5 cm AP. The external surfaces have fibrous adhesions. No parathyroid glands or lymph nodes are identified grossly. The external surface is painted with blue dye. The upper pole contains a well-delineated nodule that measures 2.7 cm SI x 2.6 cm ML x 2 cm AP and the lower pole contains a well-delineated colloid-rich lesion that measures 2.5 cm SI x 2.7 cm ML x 2 cm AP. The remainder of the thyroid tissue is slightly nodular. Sections of nodule and normal tissue are stored frozen. The remainder of the specimen is submitted as follows:

1A-J lobe, superior to inferior
1K isthmus
